

D908

Healthcare Management

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A strategic planning team at a 250-bed nonprofit hospital with a popular maternity unit is building a SWOT. The CFO recently completed a marathon. A new state law will cut Medicaid payment rates next year. Where does the law belong?

- A. Strength, because the maternity unit is popular
- B. Opportunity
- C. Threat
- D. Weakness

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

The same hospital's nurse turnover is 28%, well above the regional average. Where does this belong in the SWOT?

- A. Threat
- B. Weakness
- C. Opportunity, since every problem is an opportunity
- D. Strength

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A health system CEO who is a former Army officer wants every clinic to adopt a new scheduling model. Staff are comfortable with the current system. The organization just renovated its main lobby. Using Kotter's model, what should she do first?

- A. Form a guiding coalition of clinic leaders
- B. Create a sense of urgency by sharing data on the problems with the current model
- C. Announce a launch date and schedule training
- D. Use her command experience to mandate the change

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A new chief nursing officer at a hospital with a strong academic affiliation shares an inspiring vision, coaches staff one on one, and encourages them to question old routines. She also drives an electric car. Which leadership style is she using?

- A. Transformational
- B. Laissez-faire
- C. Servant leadership only, never transformational
- D. Transactional

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A new quality team at a clinic in a college town has met for three weeks. Members argue about roles and challenge the leader's plan. The team has a generous budget. Which stage is it in, and how should the leader respond?

- A. Forming; introduce members and explain the purpose
- B. Performing; step back, since arguing shows engagement
- C. Storming; clarify roles and help the team surface and resolve conflict
- D. Norming; let the team set its own rules

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A team has agreed-upon ground rules and shared expectations, and members help one another, but they still rely on the leader to finalize decisions. Which stage is this?

- A. Storming
- B. Norming
- C. Adjourning
- D. Performing

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

Two department directors at a hospital that recently won a patient-experience award disagree about sharing an MRI schedule. Both sets of needs are important, their working relationship matters long-term, and there is time to work it out. They went to the same graduate program. Which conflict mode fits best?

- A. Avoiding
- B. Competing
- C. Compromising
- D. Collaborating

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A sudden safety emergency requires a manager to make an immediate, unpopular decision about which unit gets the only available piece of equipment. There is no time for discussion. Which conflict mode is most appropriate?

- A. Competing
- B. Collaborating
- C. Avoiding
- D. Compromising every time, since splitting is always fairest

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A consultant at a health system with a new downtown headquarters notices “Patients First” posters in every hallway. Yet staff all take for granted that physicians’ convenience comes before patient wait times, and no one questions it. The system’s logo was redesigned last year. That unquestioned belief is which level of culture?

- A. Basic underlying assumptions
- B. Artifacts
- C. Espoused values
- D. None, since culture is only what leaders state

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

An observer at a clinic notes its open nurse stations, the staff dress code, and that meetings start exactly on time. These observations are which level of culture?

- A.** Basic underlying assumptions
- B.** Espoused values
- C.** None, because only beliefs count as culture
- D.** Artifacts

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

The board of a 25-bed critical access hospital, which recently added two new members, drafts this statement: “To be the healthiest rural county in the state by 2035.” What type of statement is this?

- A.** Measurable strategic objective, since it has a date
- B.** Mission statement
- C.** Values statement
- D.** Vision statement

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A tax-exempt nonprofit hospital known for its cardiac program wants to keep its federal exemption. Its gift shop earns about \$1 million a year. Under the ACA, what must the hospital do at least every three years?

- A.** Provide free care to every patient
- B.** Donate all gift-shop profits to charity
- C.** Conduct a community health needs assessment and adopt an implementation strategy
- D.** Begin paying federal income tax on its profits

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

A strategic planning team at a 250-bed nonprofit hospital ~~with a popular maternity unit~~ is building a SWOT. ~~The CEO recently completed a marathon.~~ A new state law will cut Medicaid payment rates next year. Where does the law belong?

Target word: SWOT category **Decides:** An outside change that hurts.

A	Camouflage	The maternity unit is unrelated to the law.
B	Inverted operation	The law hurts revenue, the opposite of an opportunity.
C	Correct	An external development with negative effects is a threat.
D	Confusion set	Weaknesses are internal; the law comes from outside.

2. Answer B Concept 1 • transfer

The same hospital's nurse turnover is 28%, well above the regional average. Where does this belong in the SWOT?

Target word: SWOT category **Decides:** An internal problem.

A	Confusion set	Threats are external; turnover is internal.
B	Correct	An internal shortfall the hospital controls is a weakness.
C	Absolute language	Opportunities are favorable external conditions.
D	Inverted operation	High turnover is a problem, not an advantage.

3. Answer B Concept 2

A health system CEO ~~who is a former Army officer~~ wants every clinic to adopt a new scheduling model. Staff are comfortable with the current system. ~~The organization just renovated its main lobby.~~ Using Kotter's model, what should she do first?

Target word: first step **Decides:** Staff do not yet feel a need to change.

A	Adjacent category	That is step two, after urgency.
B	Correct	Kotter's first step is urgency.
C	Dropped element	Skips the early steps that build commitment.
D	Camouflage	Her military background does not replace the change process.

4. Answer A Concept 3

A new chief nursing officer at a hospital ~~with a strong academic affiliation~~ shares an inspiring vision, coaches staff one on one, and encourages them to question old routines. ~~She also drives an electric car.~~ Which leadership style is she using?

Target word: leadership style **Decides:** Vision, individual coaching, intellectual challenge.

A	Correct	Those are core transformational behaviors.
B	Adjacent category	Laissez-faire is hands-off, the opposite of active coaching.
C	Absolute language	The behaviors described match transformational leadership.
D	Confusion set	Transactional leadership exchanges rewards for meeting targets.

5. Answer C Concept 4

A new quality team at a clinic ~~in a college town~~ has met for three weeks. Members argue about roles and challenge the leader's plan. ~~The team has a generous budget.~~ Which stage is it in, and how should the leader respond?

Target word: stage and response **Decides:** Open conflict over roles and direction.

A	Confusion set	Forming is polite and tentative, not argumentative.
B	Inverted operation	Treats an early conflict stage as the final high-output stage.
C	Correct	Conflict over roles and authority marks storming.
D	Adjacent category	Norming comes after conflict is resolved.

6. Answer B Concept 4 • transfer

A team has agreed-upon ground rules and shared expectations, and members help one another, but they still rely on the leader to finalize decisions. Which stage is this?

Target word: stage **Decides:** Agreed norms without full autonomy.

A	Inverted operation	Storming is conflict, which this team has moved past.
B	Correct	Shared rules and cohesion without full self-direction is norming.
C	Adjacent category	Adjourning is when the team disbands.
D	Confusion set	Performing teams work largely on their own.

7. Answer D Concept 5

Two department directors at a hospital ~~that recently won a patient experience award~~ disagree about sharing an MRI schedule. Both sets of needs are important, their working relationship matters long-term, and there is time to work it out. ~~They went to the same graduate program.~~ Which conflict mode fits best?

Target word: best conflict mode **Decides:** Important issue, key relationship, time available.

A	Adjacent category	Avoiding leaves an important issue unresolved.
B	Inverted operation	One side wins at the other's expense, harming the relationship.
C	Confusion set	Each gives up part of their needs; collaboration aims to meet both fully.
D	Correct	Collaboration fits when both concerns matter and there is time to find a full solution.

8. Answer A Concept 5 • transfer

A sudden safety emergency requires a manager to make an immediate, unpopular decision about which unit gets the only available piece of equipment. There is no time for discussion. Which conflict mode is most appropriate?

Target word: best conflict mode **Decides:** No time and high stakes.

A	Correct	Quick, decisive action is the accepted use of competing.
B	Inverted operation	Collaboration needs time the situation does not have.
C	Adjacent category	Avoiding leaves the safety issue unaddressed.
D	Absolute language	No mode fits every situation.

9. Answer A Concept 6

A consultant at a health system ~~with a new downtown headquarters~~ notices “Patients First” posters in every hallway. Yet staff all take for granted that physicians’ convenience comes before patient wait times, and no one questions it. ~~The system’s logo was redesigned last year.~~ That unquestioned belief is which level of culture?

Target word: culture level **Decides:** A taken-for-granted belief that drives behavior.

A	Correct	Unspoken beliefs everyone takes for granted are the deepest level.
B	Confusion set	Artifacts are the visible items, such as the posters.
C	Adjacent category	Espoused values are what the organization says it values.
D	Absolute language	Culture includes unspoken assumptions, not just statements.

10. Answer D Concept 6 • transfer

An observer at a clinic notes its open nurse stations, the staff dress code, and that meetings start exactly on time. These observations are which level of culture?

Target word: culture level **Decides:** Visible structures and behaviors.

A	Inverted operation	Assumptions are the invisible level, not what is observed.
B	Confusion set	Espoused values are stated goals and philosophies.
C	Absolute language	Visible artifacts are part of culture.
D	Correct	What can be seen and heard is the artifact level.

11. Answer D Concept 7

The board of a 25-bed critical access hospital, ~~which recently added two new members,~~ drafts this statement: “To be the healthiest rural county in the state by 2035.” What type of statement is this?

Target word: statement type **Decides:** Describes a desired future.

A	Dropped element	It has a date but no measure or baseline.
B	Confusion set	A mission states current purpose and whom the organization serves.
C	Adjacent category	Values describe how people should behave.
D	Correct	A future aspiration is a vision.

12. Answer C Concept 8

A tax-exempt nonprofit hospital ~~known for its cardiac program~~ wants to keep its federal exemption. ~~Its gift shop earns about \$1 million a year.~~ Under the ACA, what must the hospital do at least every three years?

Target word: ACA requirement **Decides:** Community health needs assessment.

A	Absolute language	Community benefit does not require free care for all.
B	Camouflage	Gift-shop revenue is irrelevant to the requirement.
C	Correct	The ACA requires a CHNA every three years for tax-exempt hospitals.
D	Inverted operation	That is what losing the exemption would mean.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	2	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	8	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	8	
Absolute language	Always, never, guarantee — where judgment applies.	6	
Confusion set	Every option is real; the test is exact role-matching.	9	
Camouflage	Answers a stem fact that was never the question.	3	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	SWOT: internal vs. external	Strengths and weaknesses are internal to the organization; opportunities and threats come from the outside environment.	Labeling an outside change as a weakness because it hurts.	Whether the organization controls the item or it comes from outside.
2	Kotter's change sequence	Lead change by first creating urgency, then building a guiding coalition and vision, and only later anchoring it in culture.	Starting with the rollout or training before people see why change is needed.	"What should the leader do first?" when staff are comfortable.
3	Transformational vs. transactional leadership	Transformational leaders inspire with vision, coach individuals, and invite new thinking; transactional leaders exchange rewards for meeting set goals.	Calling any effective or friendly leader transformational.	Vision and intellectual challenge versus rewards and corrections.
4	Tuckman team stages	Teams move through forming, storming, norming, performing, and adjourning, and the leader's role changes at each stage.	Treating conflict as a sign the team has failed or is already high-performing.	Arguments over roles, agreed ground rules, or self-directed output.
5	Conflict-handling modes	Collaborating fits important issues with time and a key relationship; competing fits emergencies; compromising, accommodating, and avoiding fit narrower cases.	Believing one mode, often compromise, is always best.	How much time there is, how much the issue matters, and how much the relationship matters.
6	Levels of organizational culture	Culture has visible artifacts, stated espoused values, and unspoken basic assumptions that drive real behavior.	Believing the posters and value statements are the culture.	A gap between what is said and what everyone actually does.
7	Mission, vision, and values	Mission states why the organization exists and whom it serves now; vision describes the future it aims for; values guide behavior.	Swapping mission and vision.	Present purpose versus future aspiration.
8	Nonprofit hospital community benefit	Tax-exempt hospitals must provide community benefit and, under the ACA, complete a community health needs assessment every three years with an implementation strategy.	Thinking nonprofit status only means not paying owners, or that it requires unlimited free care.	A tax-exempt hospital and a question about its obligations.

Course scope (public description): Healthcare Management explores the function, structure, and behavior of a wide variety of healthcare organizations (nonprofit, start-up hospitals, ACHE, etc.). It covers strategic planning, change management, corporate responsibility and broad managerial skill (critical thinking, adaptability, and decision-making) development within the context of healthcare. The course will focus on how organizations function and how people function within organizations (roles in the healthcare field, culture). This course prepares students to apply organizational leadership and behavior, principles of organizational culture development, techniques for building and leading teams, and conflict resolution strategies to support organizational goals. As

learners are introduced to the skills and behaviors needed to lead a healthcare organization, they will consider how they can cultivate an organizational leadership style that influences an inclusive organizational culture and fosters innovation. There are no prerequisites to the course.